This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

Japanese Encephalitis Vaccine

(Ixiaro)

Standalone PGD, separated from the combined travel document

Patient Group Direction, version 005, issued 11 September 2026. Supersedes Version 004, 10 September 2026.

This PGD was previously part of a single combined document covering Japanese encephalitis, rabies and meningococcal ACWY vaccination. That was raised by Pharmacy Plus Health on 21 August 2026: a pharmacy adopting one of the three was signing for all three, including vaccines it may not stock and in which its staff may not be trained. The three have been separated so that each can be adopted, trained for and signed off on its own merits.

Summary of Green Book chapter 20 guidance for Japanese encephalitis vaccination

UKHSA, Immunisation against infectious disease (the Green Book), Chapter 20: Japanese encephalitis, dated February 2024 (PDF hosted by NaTHNaC on TravelHealthPro and linked from the GOV.UK publication page, page last updated 6 February 2024). Summarised 10 September 2026.

This summary is part 2 of the house format for a Get Real Health PGD: what the PGD is for, then the guidance that governs the condition, then the PGD itself. It is here so that a pharmacist can hold this document against the guidance it claims to follow. It summarises the source named above and adds nothing to it.

The disease, its geography and the risk to travellers

Japanese encephalitis (JE) is a mosquito-borne viral encephalitis caused by a flavivirus and is the leading cause of childhood encephalitis in Asia.

The global incidence is unknown, but an estimated 68,000 clinical cases occur annually in 24 countries with JE risk (World Health Organization, 2019).

JE is endemic in rural areas, especially where rice growing and pig farming coexist, and epidemics occur in rural and occasionally in urban areas.

The highest transmission rates occur during and just after wet seasons when mosquitoes are most active, but seasonal patterns vary within individual countries and from year to year, and cases are also reported outside the normal seasonal period of high transmission.

The disease is not transmitted from person to person.

The incubation period is four to 14 days. Most infections are mild or asymptomatic, but approximately one in 250 infections is estimated to result in severe illness (rapid onset high fever, headache, neck stiffness, disorientation, coma, seizures).

Case fatality rates in those with symptoms can be as high as 30 percent, and permanent sequelae (intellectual, behavioural and other neurological problems such as paralysis, Parkinsonian-like movement disorders, recurrent seizures or inability to speak) are reported in 20 to 30 percent of survivors.

National immunisation campaigns and urban development in the 1960s led to the near-elimination of JE in Japan, South Korea, Singapore and Taiwan, but JE remains endemic in much of the rest of Asia, with China (excluding Taiwan) accounting for 50 percent of cases. In 2022 the first reported JE outbreak in mainland Australia was declared.

There is geographical variation in transmission periods, from all year round to seasonal. In temperate regions of Asia most cases occur in the warm season, when large epidemics can occur. In the tropics and subtropics JE can occur year-round, but transmission often intensifies during the rainy season and pre-harvest period in rice-cultivating regions.

JE is rare in British travellers, but three cases were diagnosed in 2014 and 2015 resulting in severe neurological illness and long-term sequelae. None of these travellers had received JE vaccine despite having indications for it.

Country-specific recommendations and information on the global epidemiology of JE can be found at travelhealthpro.org.uk and travax.nhs.uk.

No specific therapy is available for JE; supportive treatment can significantly reduce morbidity and mortality.

Who the chapter recommends vaccination for

The objective of JE vaccination is to protect individuals at high risk of exposure during travel or in the course of their occupation.

All travellers should undergo a careful risk assessment that takes into consideration their itinerary, season of travel, duration of stay and planned activities. The risk of JE should then be balanced against the risk of adverse events from vaccination. Travellers should also look at country-specific recommendations and reduce the risk of JE with bite avoidance.

JE vaccine is recommended for those who are going to reside in an area where JE is endemic or epidemic.

Travellers to risk areas in Asia and the Western Pacific region should be immunised if staying for a month or longer in endemic areas during the transmission season, especially if travel will include rural areas.

The chapter's flowchart (Figure 20.1) also lists frequent travel to JE endemic areas and potential exposure to JE in a laboratory as groups for whom JE vaccine is recommended provided there are no contraindications.

Other travellers with shorter exposure periods should be immunised if the risk is considered sufficient; for example, those spending a short period of time in rice fields (where the mosquito vector breeds) or close to pig farming (a reservoir host for the virus) should be considered for vaccination.

The flowchart states that JE vaccine should be considered where there is a shorter exposure period (under one month) but increased risk from the planned itinerary, location and activities (such as visiting rice fields or pig farms), or where there is travel to an endemic area with uncertainties regarding the planned itinerary, location, activities and duration of stay.

JE vaccine is not recommended for those with lower risk itineraries such as shorter stays (under one month) in towns and cities.

Immunisation is recommended for all research laboratory staff who have potential exposure to the virus; worldwide there have been 22 cases of laboratory-acquired JE virus infection.

There are no safety or efficacy data on the use of IXIARO in children under two months of age and it is not usually recommended in this age group in the UK.

The vaccine (IXIARO) and the schedule

There is currently one licensed vaccine recommended for use in the UK, IXIARO, which is licensed in the UK for individuals aged two months and older.

IXIARO is an inactivated vaccine produced in Vero cells and adsorbed onto an aluminium hydroxide adjuvant to improve its immunogenicity. As the vaccine does not contain live organisms, it cannot cause the disease against which it protects.

IXIARO is available as a 0.5 ml suspension in a pre-filled syringe (Type 1 glass) with a chlorobutyl elastomer plunger stopper.

Children aged two months to under 36 months: the licensed schedule is two doses of 0.25 ml, the first at day 0 and the second 28 days after the first dose. This is half of the standard dose, and the manufacturer's patient information leaflet should be consulted for instructions on discarding half of the vaccine. Full immunity takes up to one week to develop after the second dose.

Children aged three years to 17 years: the licensed schedule is two doses of 0.5 ml, on days 0 and 28. Full immunity takes up to one week to develop after the second dose.

Adults aged 18 years and older: the standard schedule is two doses of 0.5 ml on days 0 and 28. Alternatively, a rapid schedule of two doses on days 0 and 7 can be used.

The rapid schedule (days 0 and 7) is licensed for adults aged 18 to 64 years, and antibody responses are non-inferior to those of the standard schedule.

The rapid schedule may be used in all age groups from two months of age where there is genuinely insufficient time to complete the standard schedule before travel. It is off license for children from two months to 17 years of age and for adults aged 65 years and older because of lack of data, but there are no data to suggest that the rapid schedule would be harmful for these travellers.

With both schedules, primary immunisation should ideally be completed at least one week prior to potential exposure to JE virus.

In situations where the primary course plus initial booster has been interrupted, the schedule should be resumed and not restarted.

First booster for children (from two months) and adults under 65 years: for those at ongoing risk (such as laboratory staff and long-term travellers who expect to reside in JE endemic areas for appreciable periods of time) a booster dose should be given 12 months after primary immunisation; for other travellers a booster dose should be given within 12 to 24 months after primary immunisation, prior to potential re-exposure to JE virus.

First booster for adults aged 65 years and older: the duration of protection from the primary course is uncertain, and a booster dose at 12 months should be considered for those at continued or further risk.

Second booster for adults aged 18 to 64 years: a second booster dose (fourth dose) should be offered at 10 years to those who remain at risk.

Second booster for children: no long-term seroprotection data beyond three years after the first booster have been generated. One study in children living in a risk area estimated the duration of protection to be 9 years after the first booster dose, and no studies were identified estimating length of protection in those living outside risk areas.

Second booster for adults aged 65 years and older: the length of protection following the booster (third) dose is not known; response to the vaccine may be reduced in this age group and immunity may wane before 10 years.

IXIARO may be used as a booster for those who previously received Green Cross vaccine or Biken vaccine.

IXIARO should be given by intramuscular injection. For individuals who have a bleeding disorder it should be given by deep subcutaneous injection to reduce the risk of bleeding.

IXIARO can be given at the same time as other travel or routine vaccines. The vaccines should be given at a separate site, preferably in a different limb; if given in the same limb they should be at least 2.5 cm apart.

Equipment used for vaccination, including used vials, ampoules or partially discharged vaccines, should be disposed of at the end of a session by sealing in a proper puncture-resistant sharps box according to local authority regulations and Health Technical Memorandum 07-01.

IXIARO is available from Valneva UK Ltd.

Contraindications and precautions

There are very few individuals who cannot receive IXIARO. When there is doubt, appropriate advice should be sought from a travel health specialist.

IXIARO should not be given to those who have had a confirmed anaphylactic or serious systemic reaction to a previous dose of IXIARO vaccine.

IXIARO should not be given to those who have had a confirmed anaphylactic reaction to any component of the vaccine.

Individuals with pre-existing allergies: there is no known extra risk of hypersensitivity reactions to the IXIARO vaccine.
Bleeding disorders: IXIARO should be given by deep subcutaneous injection rather than intramuscularly to reduce the risk of bleeding.

Pregnancy and breastfeeding

As a precautionary measure, administration of IXIARO during pregnancy or lactation should be avoided. However, travellers and their medical advisers must make a risk assessment of the theoretical risks of JE vaccine in pregnancy against the potential risk of acquiring JE.
Miscarriage has been associated with JE virus infection when acquired in the first two trimesters of pregnancy.

Adverse reactions

The most common adverse reactions observed after administration of IXIARO are pain and tenderness at the injection site, headache, fatigue and myalgia.
Other commonly reported reactions are erythema, hardening, swelling and itching at the injection site, influenza-like illness, pyrexia and nausea.

Storage

Vaccines should be stored in the original packaging at +2 degrees C to +8 degrees C and protected from light.
All vaccines are sensitive to some extent to heat and cold. Heat speeds up the decline in potency of most vaccines, reducing their shelf life, and effectiveness cannot be guaranteed unless vaccines have been stored at the correct temperature.
Freezing may cause increased reactogenicity and loss of potency for some vaccines, and can cause hairline cracks in the container leading to contamination of the contents.

What this chapter does not state

The chapter does not state a post-vaccination observation period after IXIARO is given.
The chapter does not state that adrenaline must be available at the time of vaccination, and it does not refer to the management of anaphylaxis.
The chapter does not specify an injection site or needle size for the intramuscular dose; it states only that IXIARO should be given by intramuscular injection (or by deep subcutaneous injection for those with a bleeding disorder) and that co-administered vaccines should be given at separate sites.
The chapter does not state a minimum interval between the second primary dose and the first booster other than the 12 month and 12 to 24 month timings given above, and it does not give any guidance on immunosuppressed individuals.

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products supplied, as well as BNF advice and any applicable national guidance • All users must be competent in intramuscular vaccination technique and in the recognition and management of anaphylaxis • All users must be competent in travel risk assessment, including itinerary, duration, season and activity • All users must be able to apply the Green Book decision framework for who should and should not be offered this vaccine • All users must previously have used PGD's to supply or administer medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medicinal products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005

Clinical condition or situation to which this PGD applies

PGD Indication	Active immunisation against Japanese encephalitis in individuals at risk through travel or occupation. Japanese encephalitis is a mosquito-borne flavivirus infection of Asia and the western Pacific. Most infections are asymptomatic, but encephalitis carries a case fatality of around 30% and leaves lasting neurological damage in a substantial proportion of survivors. There is no specific treatment, which is why prevention matters.
Inclusion criteria	• Aged 2 months or over. Ixiaro is licensed from 2 months of age with no upper limit. • Vaccination recommended: residence in an endemic or epidemic area; a stay of one month or longer in an endemic area during the transmission season; frequent travel to endemic areas; or laboratory work with potential exposure. • Vaccination to be considered: a shorter stay with a higher risk itinerary or activity, for example rural travel, visiting rice fields or pig farming areas, or extensive outdoor and evening exposure; or an uncertain itinerary or duration within an endemic area. • Sufficient time before travel to complete the primary course, ideally with at least one week between the second dose and potential exposure. • Valid informed consent obtained, including consent to any off-label use of the rapid schedule where that applies.

	• No exclusion criteria present.
Exclusion criteria	• Under 2 months of age. • Confirmed anaphylactic or serious systemic reaction to a previous dose of Ixiaro, or to any component including the residues protamine sulphate, formaldehyde, bovine serum albumin, host cell DNA and protein, and sodium metabisulphite. • Hypersensitivity reaction following the first dose. Do not give the second dose; refer. • Acute severe febrile illness. Postpone until recovered. • Pregnancy, unless the risk of Japanese encephalitis is high and cannot be avoided. Refer for individual assessment rather than vaccinating under this PGD. • A short stay of less than one month confined to urban areas with a low risk itinerary, where vaccination is not recommended. Explain the reasoning and give bite avoidance advice rather than vaccinating.
Cautions	• Breastfeeding. Limited data. Avoid as a precaution unless the risk of exposure is significant, and record the risk assessment. • Immunosuppression. An adequate immune response may not be achieved. Counsel accordingly and consider referral for serology where the risk is high. • Bleeding disorders, thrombocytopenia or anticoagulation. Give by deep subcutaneous injection rather than intramuscularly. • Age over 65. Seroconversion is lower, around 65% compared with over 96% in adults under 50, and titres are lower. Counsel that protection may be less reliable and consider the booster at 12 months. • A history of allergy or urticaria is not in itself a reason to withhold Ixiaro. The Green Book records no known additional risk of hypersensitivity in people with pre-existing allergies. • Vaccination does not remove the need for mosquito bite avoidance, which must be counselled in every case. The mosquito that transmits Japanese encephalitis bites mainly between dusk and dawn.
Anaphylaxis and observation	• Adrenaline 1:1000 injection must be immediately available whenever a vaccine is administered under this PGD, together with a written anaphylaxis protocol consistent with current Resuscitation Council UK guidance. • All staff administering vaccines must be trained in the recognition and immediate management of anaphylaxis and must hold current basic life support training. • The patient should remain on the premises for observation after vaccination in line with the pharmacy SOP. • Procedures must be in place to prevent injury from a vasovagal faint, which is common around vaccination, particularly in adolescents.
Actions if patient is excluded or declines treatment	• Discuss the reason for exclusion with the patient and ensure they understand it. • Advise on alternative options and how these can be accessed, for example the GP, a travel clinic, or a specialist service. • Document the reason for exclusion, the advice given and the decision reached. • Inform or refer to the GP as appropriate, and where the exclusion is time critical make the urgency of that referral explicit to the

	patient.

Details of the medicine

Name, form and strength of medicine	Ixiaro (Valneva), suspension for injection in a pre-filled syringe. Inactivated, adjuvanted, Vero cell derived, strain SA14-14-2. This is the only Japanese encephalitis vaccine licensed in the UK. Record the batch number and expiry for every dose.
Legal category	POM
Storage	Store at 2°C to 8°C in the original packaging, protected from light. Do not freeze, and discard any vaccine that has been frozen. Shake before use to obtain a homogeneous white suspension; a fine white deposit with a clear supernatant on standing is normal.
Route/method of administration	• Intramuscular injection. Deltoid in older children and adults, anterolateral thigh in infants. • In bleeding disorders, thrombocytopenia or anticoagulation, give by deep subcutaneous injection instead. • May be given at the same visit as other travel vaccines at a separate site, preferably a different limb, or at least 2.5 cm apart in the same limb. Co-administration with inactivated hepatitis A vaccine and with rabies vaccine has been studied without interference.
Dose and frequency of administration	• Aged 3 years and over: 0.5 mL per dose. • Aged 2 months to under 3 years: 0.25 mL per dose. Follow the SPC instructions for discarding the excess volume using the marking on the syringe, and use a new sterile needle for the injection. • Conventional primary course, all licensed ages: two doses, day 0 and day 28. • Rapid primary course: two doses, day 0 and day 7. This is licensed only for adults aged 18 to 64. Its use in children and in adults aged 65 and over is off-label; the Green Book permits it where there is genuinely insufficient time before travel, and off-label use requires explicit documented consent. • Complete the primary course at least one week before potential exposure. • If the course is interrupted, resume it rather than restarting. A second dose given up to 11 months after the first still produces a high rate of seroconversion. • First booster: 12 to 24 months after the primary course and before re-exposure. Those at continuous risk, such as long-term residents and laboratory staff, should have it at 12 months. • Second booster: adults aged 18 to 64 at continued risk, 10 years after the first booster. Long-term data are lacking in children and in the elderly, in whom immunity may wane sooner.
Quantity to be administered and/or supplied	One dose per patient per attendance, in accordance with the schedule above.
Adverse effects	Very common: headache, myalgia, injection site pain or tenderness, and fatigue. Common: nausea, injection site redness, induration, swelling and itching, influenza-like illness and fever. Uncommon: lymphadenopathy, migraine, dizziness, vertigo, gastrointestinal upset, rash, pruritus, arthralgia, chills, malaise and raised hepatic enzymes. Rare: thrombocytopenia, paraesthesia, neuritis, syncope, palpitations, tachycardia, dyspnoea, urticaria and peripheral oedema. In children, fever, diarrhoea, influenza-like illness and irritability are the most frequent. Consult the current SPC for the full list.

Yellow card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP with whom the individual is registered • name of healthcare practitioner • name and brand of vaccine, batch number and expiry date • date of administration, dose, route and anatomical site used • where a course is involved, the dose number and the date the next dose is due • advice given, including advice given if excluded or if the patient declines • details of any adverse reaction and the action taken • that the vaccine was administered via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. The individual's GP should be informed.

Patient information

- Give the patient the manufacturer's patient information leaflet.
- Explain the schedule clearly, including the date the second dose is due and that the course must be completed at least one week before exposure.
- Counsel on mosquito bite avoidance as the primary protection: repellent containing DEET, covering up at dusk and dawn, and treated bed nets. Vaccination supplements this and does not replace it.
- Explain that no vaccine is completely protective and that any febrile illness with headache, confusion or neurological symptoms during or after travel needs urgent medical assessment, mentioning the travel history.
- Counsel on the common local and systemic reactions.
- Where the rapid schedule has been used off-label, record that this was explained and consented to.

References

- UKHSA. Immunisation against infectious disease, the Green Book, chapter 20, Japanese encephalitis. Revised 6 February 2024.
- Summary of Product Characteristics for Ixiaro, current version, electronic Medicines Compendium.
- NaTHNaC TravelHealthPro. Country information pages and Japanese encephalitis disease page.
- Resuscitation Council UK. Emergency treatment of anaphylaxis, current guidance.

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates	

Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date
11/9/26	31/7/27

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

To be completed by the adopting pharmacy: the first block is signed by that pharmacy's superintendent or clinical lead, and the second by its professional group signatory. Get Real Health signs the authorship block below, not these.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history

Version number	Change details	Date
001	New standalone PGD. Separated from the combined Japanese encephalitis, rabies and meningococcal ACWY document following the query from Pharmacy Plus Health on 21 August 2026. Content rebuilt from Green Book chapter 20 and the current Ixiaro SPC. Notable additions the combined document did not carry: the paediatric half dose of 0.25 mL below 3 years, the restriction of the day 0 and day 7 rapid schedule to adults aged 18 to 64 with off-label use elsewhere, the deep subcutaneous route in bleeding disorders, and the reduced seroconversion above 65.	21st August 2026

Vaccine safety requirements

The requirements on the following pages form part of this Patient Group Direction and must be met before any vaccine is administered under it.

Observation after vaccination
OBSERVE EVERY PATIENT FOR 15 MINUTES AFTER VACCINATION, SEATED.
Anxiety-related reactions including vasovagal syncope, hyperventilation and transient visual disturbance or paraesthesia can occur before or after any injection, and are commonest in adolescents. They are a response to the needle, not to the vaccine.
Have procedures in place to prevent injury from a faint.
Record that the observation period was completed.

Cold chain and excursions

Store at +2C to +8C in the original packaging to protect from light. Do not freeze, and discard any vaccine that has been frozen.

COLD CHAIN EXCURSION: any stock exposed to conditions outside +2C to +8C must be QUARANTINED and risk assessed in accordance with UKHSA Vaccine Incident Guidance before any further use.

Do not administer excursion stock until that assessment is complete and has been recorded.

Where the product SPC gives specific stability data for a temporary excursion, that data governs; otherwise quarantine and assess.

Disposal
Dispose of used syringes, needles, vials and any unused or reconstituted vaccine in a UN-approved puncture-resistant sharps container.
Follow local authority requirements and Health Technical Memorandum 07-01, Safe management of healthcare waste.
Never re-sheath a needle.

Consent in children and young people
Where the individual is UNDER 16, valid consent must come from a person with PARENTAL RESPONSIBILITY, or from the young person where you assess them as GILLICK COMPETENT.
Record which of the two applied. Where consent was given by a person with parental responsibility, record their name and relationship to the patient. Where the young person consented on the basis of Gillick competence, record the basis of that assessment.
A parent accompanying a child does not automatically hold parental responsibility. Ask.
Users must be competent in assessing consent in children and young people before working under this PGD.

Version and change record
Version: v005
Issued: 11 September 2026
Supersedes: Version 004, 10 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records
Version: v002
Issued: 9 September 2026
Supersedes: the previous signed version of this document.
Change: the vaccine safety requirements above were added following an estate-wide sweep of every vaccination PGD, which found that the signed documents were missing, between them, any requirement for adrenaline to be available, any anaphylaxis provision, a stated observation period, a cold chain excursion procedure, a sharps disposal provision, a batch number requirement, and any wording on consent in children and young people. The specific items added to THIS document are: Observation after vaccination, Cold chain and excursions, Disposal, Consent in children and young people.
Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Version: v003
Issued: 10 September 2026
Supersedes: Version 002, 9 September 2026
Change: the summary of the governing guidance is added as part 2 of the document, between the cover and the PGD, which is the house format for every Get Real Health PGD. It is summarised from the source it names and adds nothing to it. Nothing else in this document is altered, and this is not a clinical review of the remainder.

Version: v004
Issued: 10 September 2026
Supersedes: Version 003, 10 September 2026
Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded. No period of validity was stated.

Signed on behalf of Get Real Health

Version v005, 11 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		11 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		11 September 2026

Both authorising signatories reviewed this version together on 11 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.